

Hypercoagulation from Concomitant Administration of Tamoxifen and Warfarin

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Dataset note: This case section was extracted mechanically from the cited PMC article. No clinical facts were added.

Case presentation

A 79-year-old Caucasian woman presented to the emergency room after a motor vehicle collision. The patient suffered from multiple rib fractures, neck pain, and an open wound with a dislocated metatarsal joint. She had a blood pressure of 89/46 mmHg that improved with fluid resuscitation. She had a significant past medical history of metastatic breast cancer, nausea and vomiting, and deep venous thrombosis. Her significant past surgical history consisted of left breast lumpectomy, total abdominal hysterectomy, and ovarian cystectomy. For the past 15 months, the patient had been on tamoxifen 20 mg by mouth daily for metastatic breast cancer and on warfarin alternating between 4 mg to 6 mg by mouth daily for three years due to DVT. The patient's INR report for the past year as an outpatient showed consistent therapeutic INR values at a goal of 2-3 until one month prior to admission where her INRs were subtherapeutic. The patient was placed on treatment doses of enoxaparin in the outpatient setting due to the subtherapeutic INR values and was restarted on the same enoxaparin regimen upon hospital admission. The patient's home dose of tamoxifen and warfarin was restarted on day two of hospitalization. Her INR upon admission was 1.33 (Table 1). The patient underwent surgery for repair of her metatarsal joint on day one. On day four of hospitalization, her warfarin dose was reduced from 5 mg to 2 mg after an abrupt INR increase from 1.68 to 2.6 and her enoxaparin was placed on hold. On day five, her warfarin was held due to an increase in INR from 2.6 to 4.86. The patient's warfarin was continued to be held thereafter but her INR continued to rise. One dose of vitamin-K 2.5 mg by mouth was administered on day eight of hospitalization when her INR peaked to a maximum of 8.28. The patient's INR dropped to 1.33 the day after the oral vitamin K. Her enoxaparin was reinitiated at treatment doses (Table 1). Antibiotic coverage was expanded to cover hospital-acquired pneumonia. The patient improved and was discharged on day 12 to a skilled nursing facility on treatment doses of enoxaparin.

Table 1. Serial laboratory and medication data.

Hgb: hemoglobin; Hct: hematocrit; PT: prothrombin time; APAP: acetaminophen; SUBQ: subcutaneous; IV: intravenous

Day 1 Admission Day 2 Day 3 Day 4 Day 5 Day 6 Day 7 Day 8 Day 9 Day 10 Day 11 Day 12

Hgb (12 g/dL - 16 g/dL) 10.6 8.4 7.8 9.8 10.1 10.6 10.6 10.7 10.5 9.8 10.1 10.1

Hct (37% - 47%) 32.6 26.5 25.1 30.7 30.4 32.4 32.6 32.5 31.7 30.2 30.7 31

Platelets (150- 450 thou/cu mm) 305 233 226 172 192 236 291 317 322 305 364 378

PT (9.4 - 12.5 sec(s)) 14.5 No lab 18.5 29.2 56.5 65.3 67.4 99 14.4 14.7 17.7 22.6

INR 1.33 No lab 1.68 2.6 4.86 5.58 5.75 8.2 1.33 1.35 1.61 2.03

In-Patient Medications Administered

Warfarin Held, surgery 6 mg 5 mg 2 mg Held Held Held Held Held Held Held

Enoxaparin Held, surgery 70 mg SUBQ daily 70 mg SUBQ daily Held Held Held

70 mg SUBQ daily 70 mg SUBQ daily 70 mg SUBQ daily 70 mg SUBQ daily

Phytonadione

2.5 mg po x 1

Tamoxifen

20 mg po qday 20 mg po qday 20 mg po qday 20 mg po qday 20 mg po qday 20 mg po qday 20 mg po qday

20 mg po qday 20 mg po qday 20 mg po qday

Cefepime

1 gm IV x 1 1 gm IV q 12h 1 gm IV q 12h 1 gm IV q 12h 1 gm IV x 1

Cefazolin 1 gm IV x 1 dose 1 gm IV q8h 1 gm IV q12h 1 gm IV q12h 1 gm IV q12h 1 gm IV q12h 1 gm IV q12h 1 gm IV x 1

Clindamycin

600 mg IV x 2 600 mg IV q8h 600 mg IV q8h 600 mg IV q8h 600 mg IV x 1

Vancomycin

1 gm IV x 1

Metoclopramide

5 mg po x 2 5 mg po x 2, 5 mg IV x 1 5 mg IV x 4

Ondansetron

4mg IV x 1 4mg IV x 1 4mg IV x 1 4mg IV x 2 4 mg IV x 1 4 mg IV x 1 4 mg IV x 2 4 mg IV x 1

Promethazine

6.25 mg IV x 1 6.25 mg IV x 1

6.25 mg IV x 1

Hydrocodone/APAP

1 tab po 3 tab po 4 tab po 2 tab po

1 tab po 2 tabs po 3 tabs po 1 tab po

Total APAP daily administered

325 mg 975 mg 1300 mg 650 mg

325 mg 650 mg 975 mg 325 mg

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